AR-07 Pediatric Respiratory Distress

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Pediatric asthma/reactive airway disease, wheeze, bronchospasm, croup, stridor, bronchiolitis, or other respiratory distress. Differentiate lower-airway, upper-airway, anaphylaxis, foreign-body, and respiratory-failure pathways early.

Field flow

1. Assess	Appearance, work, air movement, SpO2, mental status, perfusion, fatigue, and position of comfort.
2. Differentiate	Wheeze, stridor, bronchiolitis, anaphylaxis, epiglottitis/bacterial tracheitis concern, foreign body, or respiratory failure.
3. Support	Least distressing effective oxygen, suction when needed, and pathway-specific bronchodilator or epinephrine.
4. Escalate	Severe/impending failure: PM-01 epinephrine when indicated, Medical-Control-directed steroid, magnesium, BVM/airway escalation, and transport.

Provider-level actions

Level	Approved actions
EMT	Comfort position, least distressing oxygen, ventilation when tiring, authorized albuterol, PM-01 for anaphylaxis, and supportive care for clear bronchiolitis.
AEMT	All EMT care plus access, repeated albuterol, weight-appropriate initial ipratropium, nebulized epinephrine for croup/stridor, and PM-01-directed IM epinephrine.
Paramedic	All prior care plus Medical-Control-directed steroid consideration, magnesium for severe asthma, monitoring, and BVM/advanced airway for impending failure.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Do not force a mask on an agitated child who is maintaining oxygenation. Use blow-by or the least distressing effective method.

• For clear bronchiolitis, use supportive care rather than routine bronchodilator or epinephrine.

• Drooling, tripod position, toxic appearance, or rapidly progressive upper-airway symptoms: do not force supine or examine the airway; notify early.

• A silent chest is a pre-arrest finding; do not delay airway support for repeated medication.

Medication reference

Medication	Approved dose / safeguard
Albuterol	Under 20 kg: 2.5 mg nebulized; 20 kg or greater: 5 mg nebulized
Ipratropium	Under 20 kg: 0.25 mg; 20 kg or greater: 0.5 mg nebulized with initial albuterol for moderate/severe bronchospasm
Nebulized epinephrine	0.5 mL/kg of 1 mg/mL, max 5 mL (5 mg), for croup/stridor
IM epinephrine	Use PM-01 Pediatric Allergic Reaction pathway for anaphylaxis or life-threatening asthma
Methylprednisolone	Medical Control only; no standing pediatric dose
Magnesium sulfate	40-50 mg/kg IV once over 15-30 min, max 2 g, for severe nonresponsive asthma

Linked pathways

PM-01 Pediatric Allergic Reaction; AR-05 Pediatric Airway; AR-06 Pediatric Failed Airway; PC-02 Pediatric Bradycardia / Poor Perfusion; AO-02 Newly Born.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.